

Title: ADHERE+ Interim Analysis 2 Safety-Treatment Discontinuations and Treatment-Emergent Adverse Event Severity

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Purpose:

This data on file packet is the reference for the following information:

ADHERE+ is a phase 2, open-label extension trial to investigate the long-term safety, tolerability, and efficacy of efgartigimod alfa and recombinant human hyaluronidase PH20 for subcutaneous injection (efgartigimod rHuPH20 SC) in adult patients with chronic inflammatory demyelinating polyneuropathy (CIDP).

ADHERE+ interim analysis 2 (IA2; cutoff date: Feb 16, 2024) provides long-term safety data that continue to contribute to the overall safety profile of efgartigimod rHuPH20 SC. However, extensive data on treatment discontinuations, as well as the severity of treatment-emergent adverse events (TEAEs), are not publicly available.

These data are required to support statements on the number of/reasons for treatment discontinuations and the severity of TEAEs being experienced in ADHERE+.

Use(s):

Please describe the manner in which the following data will be utilized (e.g. multiple assets, multiple agencies, etc.):

These data may be used in the following example medical materials, including but not limited to:

- Medical science liaison (MSL) field materials
- Medical Information documents
- Field readiness guide
- Advisory boards
- Symposia
- Health economic and outcomes research (HEOR) materials

These data may be used in the following example commercial materials, including but not limited to:

- Speaker decks
- Pre-approval information exchange (PIE)
- Payer value proposition (PVP) decks
- Frequently asked questions (FAQs)
- HCP marketing materials
- Patient marketing materials

Content:

Background on Presented Safety Data

The data presented below are from an interim analysis of ADHERE+ (IA2, cutoff date: Feb 16, 2024). Unless otherwise specified, disposition, participant population, and safety analyses were performed on the safety analysis set.

As of IA2, a total of 228 participants received a dose of efgartigimod rHuPH20 SC in ADHERE+. The total participants included those who entered ADHERE+ from the ADHERE run-in/Stage A directly (i.e., those who received efgartigimod rHuPH20 SC only in Stage A) or from ADHERE

Stage B (i.e., those who received efgartigimod rHuPH20 SC or placebo in Stage B). Safety data presented below focus on the “Total” group unless otherwise specified.

Treatment Discontinuations During ADHERE+ as of IA2 (Safety Analysis Set)

Of the 228 total participants who received a dose of efgartigimod rHuPH20 SC in ADHERE+, 51 (22.4%) discontinued treatment as of IA2. The reasons for treatment discontinuation were:

- Adverse event (n=17, 7.5%)
- Withdrawal by participant (n=13, 5.7%)
- Lack of efficacy (n=7, 3.1%)
- Other (n=7, 3.1%)
- Lost to follow-up (n=3, 1.3%)
- Death (n=1, 0.4%)
- Physician decision (n=1, 0.4%)
- Required prohibited medication (n=1, 0.4%)
- Sponsor decision (n=1, 0.4%)

Treatment discontinuations during ADHERE+ as of IA2 are provided in Table 14.1.1.9 below.

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TABLE 14.1.1.9: TREATMENT DISCONTINUATION

ANALYSIS SET: SAFETY

	RI/STAGE A ONLY (N=29)	STAGE B - EFG PH20 SC (N=99)	STAGE B - PBO PH20 SC (N=100)	TOTAL (N=228)
NUMBER OF PARTICIPANTS	n (%)	n (%)	n (%)	n (%)
OVERALL				
COMPLETED	0	2 (2.0)	1 (1.0)	3 (1.3)
DISCONTINUED	5 (17.2)	21 (21.2)	25 (25.0)	51 (22.4)
ADVERSE EVENT	1 (3.4)	7 (7.1)	9 (9.0)	17 (7.5)
DEATH	0	0	1 (1.0)	1 (0.4)
LACK OF EFFICACY	0	4 (4.0)	3 (3.0)	7 (3.1)
LOST TO FOLLOW-UP	1 (3.4)	1 (1.0)	1 (1.0)	3 (1.3)
PHYSICIAN DECISION	1 (3.4)	0	0	1 (0.4)
REQUIRES PROHIBITED MEDICATION	0	0	1 (1.0)	1 (0.4)
SPONSOR DECISION	0	1 (1.0)	0	1 (0.4)
WITHDRAWAL BY SUBJECT	2 (6.9)	5 (5.1)	6 (6.0)	13 (5.7)
OTHER	0	3 (3.0)	4 (4.0)	7 (3.1)
ONGOING	24 (82.8)	76 (76.8)	74 (74.0)	174 (76.3)
CYCLE 1				
COMPLETED	1 (3.4)	45 (45.5)	50 (50.0)	96 (42.1)
DISCONTINUED	5 (17.2)	19 (19.2)	20 (20.0)	44 (19.3)
ADVERSE EVENT	1 (3.4)	7 (7.1)	8 (8.0)	16 (7.0)
LACK OF EFFICACY	0	3 (3.0)	3 (3.0)	6 (2.6)
LOST TO FOLLOW-UP	1 (3.4)	1 (1.0)	1 (1.0)	3 (1.3)
PHYSICIAN DECISION	1 (3.4)	0	0	1 (0.4)
REQUIRES PROHIBITED MEDICATION	0	0	1 (1.0)	1 (0.4)
SPONSOR DECISION	0	1 (1.0)	0	1 (0.4)
WITHDRAWAL BY SUBJECT	2 (6.9)	5 (5.1)	6 (6.0)	13 (5.7)
OTHER	0	2 (2.0)	1 (1.0)	3 (1.3)
ONGOING	23 (79.3)	35 (35.4)	30 (30.0)	88 (38.6)

THE DENOMINATOR FOR THE PERCENTAGE CALCULATIONS IS THE TOTAL NUMBER OF PARTICIPANTS IN THE SAFETY ANALYSIS SET PER TREATMENT GROUP.

Grade ≥3 TEAEs by Medical Dictionary for Regulatory Activities (MedDRA) System Organ Class and Preferred Term for ADHERE+ as of IA2 (Safety Analysis Set)

Of the 228 total participants who received a dose of efgartigimod rHuPH20 SC in ADHERE+, 41 (18.0%) experienced a grade ≥3 TEAE as of IA2. There were 263.04 participant years of follow-up (PYFU) as of IA2, with 81 events occurring in 228 participants (rate of 30.8 events per 100 PYFU).

Grade ≥3 TEAEs by MedDRA system organ class and preferred term for ADHERE+ as of IA2 are provided in Table 14.3.1.5 below.

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TABLE 14.3.1.5: GRADE 3 OR MORE TREATMENT-EMERGENT ADVERSE EVENTS BY MEDDRA SYSTEM ORGAN CLASS AND PREFERRED TERM

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ANALYSIS SET: SAFETY

ANALYSIS PERIOD = TREATMENT + FOLLOW-UP

SYSTEM ORGAN CLASS AND PREFERRED TERM	RI/STAGE A ONLY (N=29, PYFU =22.9)				STAGE B - EFG PH20 SC (N=99, PYFU =113.43)				STAGE B - PBO PH20 SC (N=100, PYFU =126.72)				TOTAL (N=228, PYFU =263.04)			
	n	(%)	m	ER	n	(%)	m	ER	n	(%)	m	ER	n	(%)	m	ER
ANY GRADE 3 OR MORE TEAE	4	(13.8)	9	39.3	15	(15.2)	23	20.3	22	(22.0)	49	38.7	41	(18.0)	81	30.8
BLOOD AND LYMPHATIC SYSTEM DISORDERS	0				0				3	(3.0)	4	3.2	3	(1.3)	4	1.5
ANAEMIA	0				0				1	(1.0)	1	0.8	1	(0.4)	1	0.4
BLOOD LOSS ANAEMIA	0				0				1	(1.0)	1	0.8	1	(0.4)	1	0.4
COAGULOPATHY	0				0				1	(1.0)	1	0.8	1	(0.4)	1	0.4
LYMPHADENITIS	0				0				1	(1.0)	1	0.8	1	(0.4)	1	0.4
CARDIAC DISORDERS	1	(3.4)	1	4.4	1	(1.0)	1	0.9	2	(2.0)	5	3.9	4	(1.8)	7	2.7
ANGINA UNSTABLE	0				1	(1.0)	1	0.9	0				1	(0.4)	1	0.4
AORTIC VALVE STENOSIS	0				0				1	(1.0)	1	0.8	1	(0.4)	1	0.4
ATRIAL FIBRILLATION	1	(3.4)	1	4.4	0				2	(2.0)	2	1.6	3	(1.3)	3	1.1
ATRIAL FLUTTER	0				0				1	(1.0)	1	0.8	1	(0.4)	1	0.4
CARDIAC ARREST	0				0				1	(1.0)	1	0.8	1	(0.4)	1	0.4
EAR AND LABYRINTH DISORDERS	0				0				1	(1.0)	1	0.8	1	(0.4)	1	0.4
DEAFNESS NEUROSENSORY	0				0				1	(1.0)	1	0.8	1	(0.4)	1	0.4
GASTROINTESTINAL DISORDERS	0				1	(1.0)	1	0.9	0				1	(0.4)	1	0.4
INGUINAL HERNIA	0				1	(1.0)	1	0.9	0				1	(0.4)	1	0.4

n = NUMBER OF PARTICIPANTS WITH EVENT; m = NUMBER OF EVENTS; TEAE = TREATMENT-EMERGENT ADVERSE EVENT; PYFU = PARTICIPANT YEARS OF FOLLOW-UP (=SUM OF FOLLOW-UP TIME OF ALL PARTICIPANTS EXPRESSED IN YEARS, PER TREATMENT GROUP); ER = EVENT RATE PER 100 PARTICIPANT-YEAR OF FOLLOW-UP (= m*100/PYFU).

CODING IS BASED ON MEDDRA VERSION 25.1 (SEP2022).

THE DENOMINATOR FOR THE PERCENTAGE CALCULATIONS IS N: THE TOTAL NUMBER OF PARTICIPANTS IN THE SAFETY ANALYSIS SET PER TREATMENT GROUP.

ADVERSE EVENTS ARE GRADED BY THE INVESTIGATOR USING THE NCI CTCAE VERSION 5.0 GRADING SCALE.

ADVERSE EVENTS THAT STARTED DURING FOLLOW-UP (IF ANY) ARE INCLUDED IN THIS TABLE.

References:

- Top-Line Results Report-ARGX-113-1902-IA206. Aug 2024.
- ARGX-113-1902 IA2 Table 14.1.1.9: Treatment Discontinuation
- ARGX-113-1902 IA2 Table 14.3.1.5: Grade 3 or More Treatment-Emergent Adverse Events By MedDRA System Organ Class and Preferred Term

Talking Points:

ADHERE+ IA2 Treatment Discontinuations

- Of the 228 participants who received a dose of efgartigimod rHuPH20 SC in ADHERE+, 51 (22.4%) discontinued treatment as of the second interim analysis (IA2).
 - The reasons for treatment discontinuation were adverse event (n=17), withdrawal by participant (n=13), lack of efficacy (n=7), other (n=7), lost to follow-up (n=3), death (n=1), physician decision (n=1), required prohibited medication (n=1), and sponsor decision (n=1).

Grade ≥3 TEAEs by MedDRA System Organ Class and Preferred Term for ADHERE+ IA2

- In the 228 participants who received a dose of efgartigimod rHuPH20 SC in ADHERE+, most TEAEs were mild to moderate in severity as of the second interim analysis (IA2).
 - Forty-one (18.0%) of the 228 participants who received a dose of efgartigimod rHuPH20 SC in ADHERE+ experienced a grade ≥3 TEAE as of IA2.
 - There were 263.04 PYFU as of IA2, with 81 events occurring in 228 participants (rate of 30.8 events per 100 PYFU).

Checklist for additional requirements to appropriately describe analyses: X Type of analysis-Interim analysis from ADHERE+ (phase 2 open-label extension trial) X Analysis set-Safety analysis set ☐ Mean at baseline ☐ Confounders, if applicable X Limitations of analysis-ADHERE+ was a single-arm, open-label extension trial and used descriptive statistics. Therefore, data should be interpreted with caution and conclusions cannot be drawn.	
Approval	
U.S. Medical Leadership	Name: Josh Bryson Signed by: <i>Josh Bryson</i>
	Date: 01-May-2025 Signer Name: Josh Bryson Signing Reason: I approve this document Signing Time: 01-May-2025 6:57:11 PM CEST B283298A5DDF4E3296EFD8FF1DD38FD3
EMA Medical Leadership	Name: Roland Derwand Signed by: <i>Roland Derwand</i>
	Date: 04-May-2025 Signer Name: Roland Derwand Signing Reason: I approve this document Signing Time: 04-May-2025 12:09:17 PM CEST DE8D73E55BA14E4DB575A89F18310A5A
Chief Medical Officer	Name: Luc Truyen Signed by: <i>Luc Truyen</i>
	Date: 02-May-2025 Signer Name: Luc Truyen Signing Reason: I approve this document Signing Time: 02-May-2025 8:28:04 AM CEST 176A66BDE16F435993316BB8F181D07